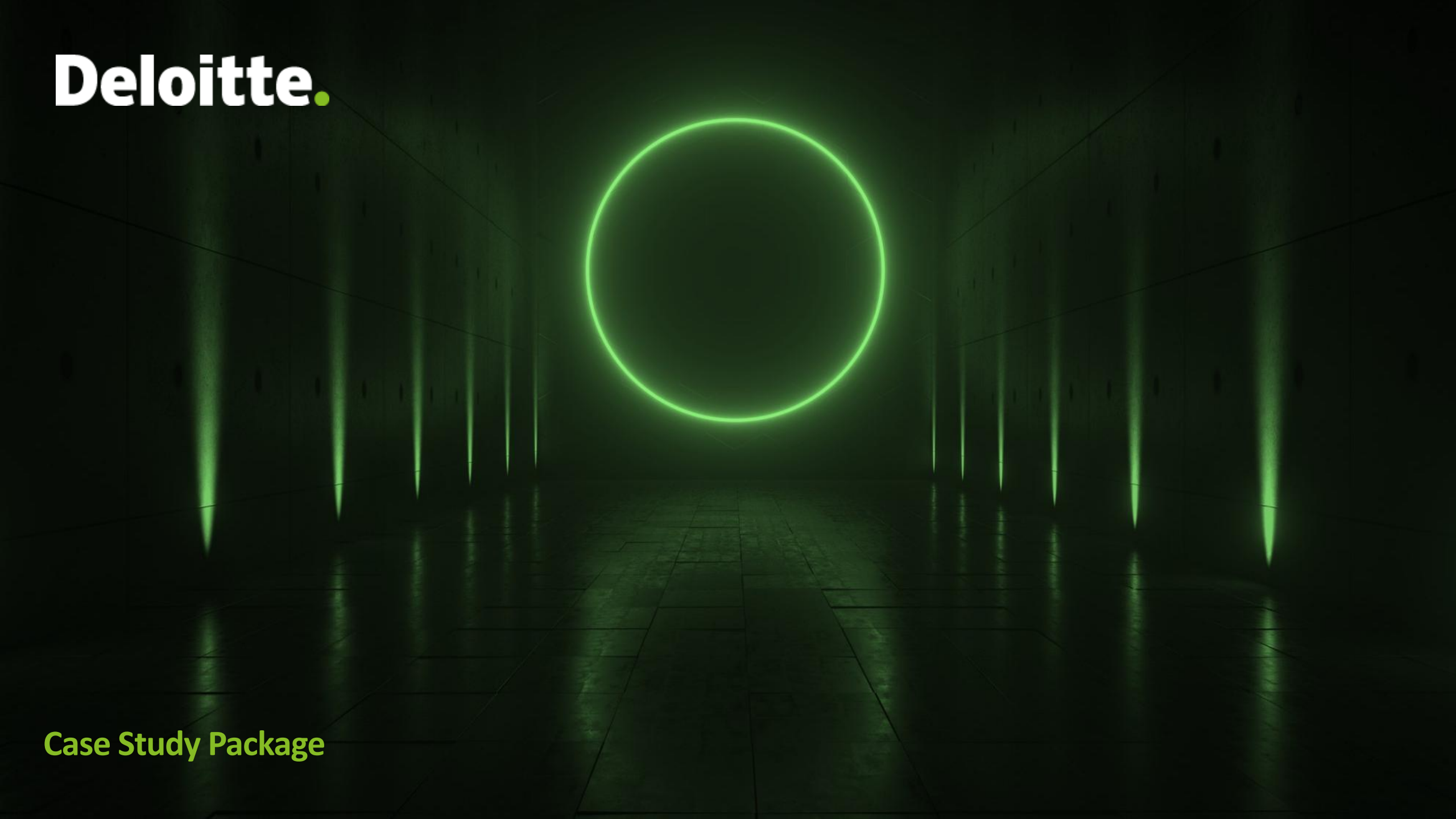

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Case Study Package

Important Case Convention

Fictional case data

All company names, financial information and operating details in this case are fictional.

The information provided in the case should be treated as the source of truth for all company-specific analysis.

Research boundaries

You may use publicly available information to understand the relevant industry, customer needs, market trends and general benchmarks.

Do not attempt to identify or research the fictional companies themselves.

Where information is not provided, state reasonable assumptions clearly and explain how they affect your recommendation.

Use of AI

Feel free to use AI to challenge your thinking, structure the analysis and test assumptions. **The judgment and the final recommendation must be your own.**

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Project Orchard: Case Overview

Business Context
Elena Marsh, CEO of Alderbrook Beverage Company, was evaluating growth options as its core categories slowed. Two potential acquisition targets had emerged: Brightwater Botanicals, a functional-soda brand valued at $450M by its private-equity minority investor, and Marrow Lane, a cold-brew coffee and tea producer seeking $170M.

About Alderbrook
Alderbrook produces juices, iced teas, sparkling waters and hydration drinks. FY25 revenue was $1.8B, growing at a rate of 2%, with an 11% operating margin and a 15% EBITDA margin (about $270M). It operates four bottling plants running at 68% average utilization and a direct-store-delivery network spanning 60 distribution centers, 1,900 drivers and 110,000 outlets. It holds $260M in cash, with net debt of 1.2x EBITDA (about $324M). The board is comfortable with net debt up to 2.5x EBITDA; lender covenants cap it at 3.5x. Juice sales were declining; Iced tea was flat and sparkling-water growth faced strong national competitors. Meanwhile, the US functional and better-for-you beverage market reached approximately $16B, with projected annual growth of 12-15% through 2030. Alderbrook's prebiotic soda, Vital, had gained little traction after two years and remained reliant on promotions. Alderbrook still plans new products from annual consumer panels and has no AI or data-science team on the commercial side.

Target 1: Brightwater Botanicals
Founded in 2019 in Boulder, Colorado, Brightwater sold prebiotic and botanical sparkling drinks in flavors that changed seasonally, and had grown revenue to $95 million, up 55% YOY, on a 44% gross margin and a small operating loss. Sales are concentrated in natural and specialty grocery, with limited conventional-grocery coverage and 10% online. It relies on regional distributors charging 25-30% of shelf price. It relies on three co-packers for manufacturing. Its 40-person team includes 5 specialists using AI tools to track social media, retailer data and review sites for flavor and occasion trends, cutting concept-to-shelf time to 90 days. The founder would remain through a transition but is concerned about preserving the brand’s identity.

Target 2: Marrow Lane
Founded in 2012 in Richmond, Virginia, Marrow Lane sells ready-to-drink cold-brew coffee and tea across the mid-Atlantic and Southeast. Revenue was $48M, growing 14%, with a 40% gross margin and a 9% operating margin. Its convenience, foodservice and regional-grocery channels overlap with Alderbrook’s network. It owns one plant operating at 70% capacity. Both founders seek a full exit, with no clear management successor.

The Board’s View
The board has asked management to assess the options ahead of its next meeting in six weeks. Directors have raised concerns about Alderbrook's two underperforming prior acquisitions and whether a Brightwater distribution partnership could offer comparable benefits at lower cost. They also see potential in Alderbrook's weekly outlet-level sales data, currently used mainly for route planning. Management must determine which path best addresses the company's growth and capability gaps.

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Project Orchard: Data Exhibit

Company Overview

FY25 Overview	Alderbrook Beverage	Brightwater Botanicals	Marrow Lane
Revenue (Growth)	$1.8B (+2%)	$95M (+55%)	$48M (+14%)
Gross margin	38%	44%	40%
EBITDA margin	15%	(2%)	13%
Operating margin	11%	(3%)	9%
Channels	Grocery, convenience, drug stores, foodservice via own DSD network; ~110,000 outlets	Natural/specialty grocery 65%, conventional grocery 25%, online 10%	Convenience and foodservice ~70%, regional grocery ~30%
Workforce	3,100 employees; 1,900 route drivers	40 employees; founder-led	110 employees; founder-led

Alderbrook portfolio snapshot

- Juice 38% of revenue, declining 3% a year; iced tea 24%, flat; sparkling water 26%, +7%; hydration and other (including Vital) 12%, +11%
- Alderbrook Vital (prebiotic soda, launched 2024): $22M revenue; 60% of volume sold on promotion; 14 months concept-to-shelf
- Manufacturing: four bottling plants at 68% average utilization; two have can lines that can run Brightwater's formats
- Board ambition for FY29: 5-6% annual revenue growth; at least 12% of revenue from functional and better-for-you drinks; operating margin of at least 11%
- Two prior acquisitions (2016, 2019; $40M and $65M): both brands now below acquisition-year revenue

Target Performance Indicators

Indicator	Brightwater	Marrow Lane
Repeat purchase	46% within 12 months	Not provided
Sales velocity vs. category average	Natural: 2.1x Conventional: 0.8x	Not provided
Manufacturing	Co-packing: 18% cost premium[1]	Owned plant: 70% utilization
Standalone FY29 revenue forecast[2]	$260-300M	$75-85M

Strategic Options

Option	Investment assumptions	Timing / terms
Acquire Brightwater	$450M indicated price ~$12M transaction costs ~$30M integration, retention and channel-launch investment	Implementation investment over two years
Acquire Marrow Lane	$170M indicated price ~$6M transaction costs Integration investment not specified	Implementation timing to be assessed
Distribution partnership	No equity purchase Commercial terms to be negotiated	Conventional-channel exclusivity; no brand control; possible sale to a competitor
Relaunch Alderbrook Vital	~$50M over three years for innovation, marketing and a dedicated team	24-36 months to meaningful scale

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[1] Versus Alderbrook's in-house cost per case for comparable formats. [2] Target management estimates. All figures are fictional and are the source of truth for this case; state assumptions for missing information.

Project Orchard: The Ask

Prepare a recommendation to Elena Marsh and Alderbrook's board that addresses the following:

1. What is limiting Alderbrook's growth, and how should changing consumer and retailer demand shape its strategic priorities?
2. Which path should Alderbrook pursue: acquiring Brightwater or Marrow Lane, forming a distribution partnership, developing its own offering, or another approach? Compare the options and explain your recommendation.
3. What value could your recommended path create, and would it justify the investment required? If recommending an acquisition, consider the indicated price, potential synergies and how the deal would be funded within Alderbrook's leverage limits.
4. What would it take to execute your recommendation successfully? Identify the key questions that must be answered before going ahead with your recommendation.
5. What role could AI and data play in your recommendation, for example in speeding up product development or using Alderbrook's outlet-level sales data, and what capabilities would Alderbrook need to build or acquire?

Industry research guidance

You may use external research to inform your perspective on the US beverage market, including:

- Demand and consumer preferences for functional, low-sugar and better-for-you beverages
- Beverage distribution models, including direct-store-delivery and third-party distributors
- Expanding emerging brands from specialty retail into mainstream grocery and convenience channels
- Transaction trends and valuation approaches for beverage brands
- Value creation, brand protection and integration risks in acquisitions
- Use real companies only as industry reference points. Assess Alderbrook, Brightwater and Marrow Lane using the facts provided in this case

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Project Silver: Case Overview

Business Context
Bellwood Health System is a US investor-owned health system based in Omaha, Nebraska, operating three hospitals and 46 outpatient and specialty clinics across the Upper Midwest. It provides primary and urgent care, specialty care, and inpatient and emergency services. Its specialties include orthopedics, cardiology and behavioral health. Over three decades, Bellwood has built its reputation on providing consistent care to local communities.

Patient and Referral Experience
Bellwood has received mixed feedback from patients and referring physicians. Some report concerns about appointment availability, scheduling and communication, while patients have also raised questions about billing clarity. Experiences vary across services and locations. Management has not established which issues matter most to different patient and referral groups, or how strongly they influence where care is sought.

Growth with a Profitability Gap
Bellwood generated $2.3B in FY25 revenue, following approximately 3% annual growth over the past five years. Its 4% operating margin remains three percentage points below the board's 7% target, established three years ago. Performance varies across services and locations: some clinics face appointment backlogs, while others have unused capacity. Differences in staffing, scheduling and administrative practices further complicate management's assessment of the profitability gap.

Technology and AI
Bellwood runs one electronic health record across all sites, but appointments are still booked by phone, clinic by clinic, and referrals are tracked by hand. A six-month pilot of AI note-taking in two primary-care clinics cut clinicians' documentation time by about 25%. The board wants to know where AI and automation could improve access and lower cost without putting clinical quality or patient trust at risk.

Choices for the Existing Business and Future Growth
Bellwood's orthopedics and musculoskeletal service line earns a 10% operating margin and has 7% annual revenue growth over the past three years, outperforming the system overall. A neighboring metro market, approximately ninety minutes from Bellwood's core service area and similar in population to its core market, is one potential expansion opportunity. Bellwood estimates that fewer than 5% of specialty referrals originating in that market are directed to Bellwood providers for specialty care. However, expansion would require investment and sufficient clinical capacity, while opportunities to improve access and performance within the existing network remain unresolved.

The Board's Mandate
The board has asked management to develop an 18-month plan that establishes a credible path toward a 7% operating margin while sustaining growth. Management must determine which patient and referring physician needs and services to prioritize, where to invest and what changes to make to Bellwood's operating model and footprint. Funding is limited to $35M of new investment over the 18 months, and any proposed changes, including any use of AI, must preserve clinical quality, patient access and continuity of care.

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Project Silver: Data Exhibit

Service-line economics

Service line	Revenue[1] (in $M)	Operating margin
Primary and Urgent care	400	1%
Orthopedics	300	10%
Other specialty care	500	5%
Inpatient / emergency	1,100	3%
Total	**2,300**	**4%**

Capacity across two orthopedic clinics, FY25

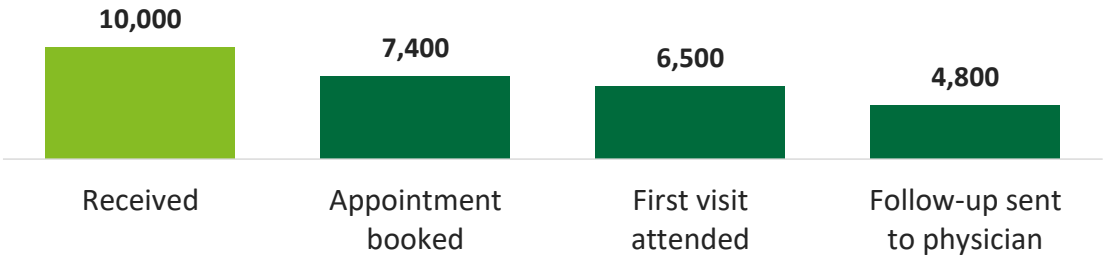
Clinic	Annual staffed slots	Utilization	Median wait
Core	20,000	94%	28 days
Suburban	20,000	70%	11 days

Utilization = visits / staffed slots. Wait = days from request to offered new appointment. A slot is one staffed appointment (one visit). New and follow-up visits use the same slots

Financial planning inputs

- **Annual spend**: contract labor $120M (FY23: $85M); nonclinical $180M; admin / support $200M
- **Investment capacity**: $35M initial funds, hard cap for 18 months (no new debt or equity); savings reinvestable; clinic reconfiguration $2-4M per site
- **New multi-specialty clinic benchmark:** $12-18M upfront; $3-5M added annual fixed costs; 9-12 month clinician recruitment
- **AI and automation**: AI-assisted scheduling and referral tracking costs $4-6M to implement and about $1.5M a year to run, with a 9–12-month rollout. AI note-taking costs about $3,000 per clinician a year; the pilot cut documentation time by about 25%. About 30% of Administration / Support spend is scheduling, referral processing, billing and coding work.

Specialty referral follow-through - Representative FY25 sample of 10,000 referrals received by Bellwood in its core market, across all specialties, each followed for 60 days. Bellwood receives about 90,000 specialty referrals a year in its core market. A split by specialty is not available.



Neighboring metro market

- 180,000 specialty referrals a year, all specialties
- Bellwood receives 7,200 (4%); two other established systems together receive 70%

Incremental economics

- **Net revenue per specialty episode**: $1,500-2,500, incl. related visits, tests and procedures within 90 days of first visit; uses about three orthopedic clinic slots
- **Contribution margin**: 25-35% before new fixed costs

Patient survey (n=200 per service group)

- **Primary / urgent care:** Convenient hours (66%); rapid access (58%)
- **Orthopedics:** Clinical expertise (78%); rapid access (62%)
- **Other specialty care:** Clinical expertise (70%); billing clarity (54%)

Patients selected two care priorities and top two responses are shown above

Referring physician survey (n=100)

- 82% rate clinical care positively
- 42% report difficulty with appointment booking
- 36% report gaps in follow-up communication

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[1] Revenue and costs are allocated once across the four categories shown. All figures are fictional and are the source of truth for this case; state assumptions for missing information.

Project Silver: The Ask

Prepare a recommendation to Bellwood's board that addresses the following:

1. What is driving Bellwood's profitability gap, and which patient and referring-physician segments, and which of their needs, should it prioritize?
2. Which growth opportunities and changes to service delivery, including the use of AI and automation, should Bellwood pursue for its priority segments, and why?
3. Where could AI and automation improve patient access and lower cost, which uses should Bellwood fund within the $35M limit, and what safeguards would protect clinical quality and patient trust?
4. What financial impact could the plan deliver, and how should Bellwood fund it within the $35M limit? State expected benefits, investment costs and key assumptions while doing the analysis.
5. Outline an 18-month plan roadmap and governance approach for balancing growth investment with the performance-improvement actions you've identified, including safeguards for any use of AI.

A segment is a group of patients or referring physicians (for example, orthopedic patients). A need is what that group values when choosing where to get care (for example, rapid access).

Industry research guidance

You may use external research to inform your perspective on US health systems, including:

- Patient access, scheduling and referral management in multi-site health systems
- What patients and referring physicians value when choosing where to send or seek specialty care
- Operating-margin drivers and improvement levers: contract labor, purchasing and administrative cost
- Growth of outpatient specialty care, including orthopedics, and the economics of opening clinics in a new market
- Use of AI and automation in scheduling, clinical documentation and billing, and the safeguards it needs
- Use real health systems only as industry reference points. Assess Bellwood using the facts provided in this case

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